

Mouth basics: hygiene, gums, and breath

Week 6, Lecture 1 · Looksmaxxing: Evidence-Based Softmaxxing

Autumn 2026 · Dental, oral aesthetics, and the jawline conversation

What we'll cover

- What plaque biofilm is and why it matters to your gum health
- The ADA-recommended brushing technique, step by step
- What the Cochrane 2019 systematic review actually found about flossing
- Interdental brushes versus floss: which performs better and why
- Tongue cleaning and the microbial sources of bad breath
- How to stack these habits into your existing morning and bedtime routines

Plaque biofilm: the enemy you cannot
see

What plaque biofilm is

Dental plaque is not just food residue. It is a structured biofilm.

- Bacteria colonize the tooth surface within hours of cleaning
- They secrete a polysaccharide matrix that anchors them in place
- The biofilm protects bacteria from saliva and low-concentration antimicrobials
- Left undisturbed for 48-72 hours, biofilm begins to mineralize into calculus (tartar)
- Calculus cannot be removed by brushing; it requires professional scaling

Mechanical disruption every 12-24 hours is the only reliable way to prevent buildup.

Why brushing angle matters

The ADA recommends a 45-degree angle to the gumline for one specific reason.

- At 45 degrees, bristle tips reach into the gingival sulcus (the small pocket between tooth and gum)
- This is where supragingival plaque accumulates and where gingivitis starts
- Horizontal scrubbing misses the sulcus and can abrade enamel over years
- Short tooth-wide back-and-forth strokes, not long sweeps

"Position your toothbrush at a 45-degree angle to the gums, and move it back and forth in short (tooth-wide) strokes."

American Dental Association (MouthHealthy.org, 2023)

The two-minute rule

Most people brush for 45 seconds. Two minutes is the ADA recommendation.

- Four quadrants: 30 seconds each
- Outer surfaces, inner surfaces, chewing surfaces
- Soft bristles only: medium and hard bristles increase enamel abrasion risk
- Replace the brush when bristles splay, or every 3-4 months
- Powered toothbrushes are not required but can improve technique consistency for some users

(ADA MouthHealthy.org, 2023)

The flossing evidence: what we actually know

What the Cochrane review found

The 2019 Cochrane systematic review by Worthington et al. is the most rigorous available synthesis on interdental cleaning.

Main findings:

- Flossing plus brushing may reduce gingivitis and plaque more than brushing alone
- Evidence is rated **low to very low certainty**
- Most trials had high risk of bias; results were inconsistent across studies
- No evidence that interdental cleaning reduces severe periodontitis or interproximal caries

Low certainty does not mean the effect is absent. It means the existing trials are not well-designed enough to be confident.

(Worthington et al., Cochrane, 2019)

Interdental brushes vs floss

The same Cochrane review found a notable secondary result.

- Interdental brushes may be more effective than floss at reducing gingivitis
- Interdental brushes clean more of the tooth surface in proximal contacts
- They are easier to use correctly than floss for most people
- Floss remains appropriate where the contact is too tight for an interdental brush

Practical takeaway: if you find interdental brushes easier to use consistently, the evidence supports using them. Compliance matters more than the tool.

(Worthington et al., Cochrane, 2019)

Tongue cleaning and halitosis

Where bad breath actually comes from

Halitosis has two primary oral sources.

1. **Dorsal tongue surface:** the largest reservoir of volatile sulfur compounds (VSCs) in the mouth. Bacteria in tongue coating metabolize protein debris and produce hydrogen sulfide and methyl mercaptan.
2. **Subgingival plaque:** bacteria in gum pockets produce VSCs continuously; untreated gingivitis significantly worsens breath.

Minor contributors: dry mouth (reduced saliva flow), diet (onion, garlic metabolites persist systemically), and systemic conditions (acid reflux, nasal drip).

Mouthwash masks odor for 30-60 minutes; it does not address the biofilm source.

Tongue scraping: mechanism and method

A tongue scraper is more effective than brushing the tongue surface.

- Brushing the tongue dislodges the surface layer but disperses bacteria laterally
- A scraper removes the entire coated layer in 3-5 strokes from back to front
- The gag reflex is less likely because the scraper moves toward the tip, not toward the throat
- Once daily, integrated into the bedtime routine, is sufficient for most people

Note: tongue cleaning reduces but does not eliminate VSCs. If halitosis persists after consistent oral hygiene, see a dentist to rule out periodontal disease.

Habit stacking your oral routine

Apply the habit-stacking framework from week 2: anchor new steps to existing ones.

Morning anchor (after waking):

- Brush 2 min (45-degree angle, short strokes) with fluoride toothpaste
- Tongue scrape: 3-5 strokes
- Do not rinse immediately; residual fluoride continues working

Bedtime anchor (last thing before sleep):

- Interdental cleaning first (floss or interdental brush): this loosens biofilm
- Brush 2 min: removes what interdental cleaning loosened
- Tongue scrape if morning-only felt insufficient

Take-aways

1. Plaque biofilm is a structured bacterial matrix that reforms within hours; mechanical disruption twice daily is the only way to prevent it from mineralizing.
2. The ADA recommends a 45-degree angle to the gumline and short tooth-wide strokes; this technique targets the gingival sulcus where gingivitis begins.
3. The Cochrane 2019 review found low-to-very-low certainty evidence that interdental cleaning reduces gingivitis; this is a reason to be honest about the evidence, not a reason to skip it.
4. Interdental brushes may outperform floss for gingivitis reduction; use whichever you will do consistently.
5. The tongue dorsum is the primary source of halitosis; a scraper removes the coated layer more effectively than a toothbrush.

"Use of floss or interdental brushes in addition to toothbrushing may reduce gingivitis or plaque, or both, more than toothbrushing alone, though evidence is low-to-very-low certainty. Interdental brushes may be more effective than floss."

Worthington HV, MacDonald L, Pockleperovic Pericic T, et al.

Cochrane Systematic Review, 2019

What's next

- **Reading:** “Dental, whitening, and the jawline myths” at [/c/looksmaxxing-26au/readings/wk06](#) (covers all week 6 content)
- **Lecture 2 (this week):** Whitening pathways, orthodontics, mewing, masseter hypertrophy, and body-recomposition effects on the jawline
- **Section this week:** 24-hour oral hygiene cycle worksheet with partner defense
- **HW2 due this week:** Two-week skincare and grooming log